

Impact

Approach

Medications

Follow-up

Side Effects

Cases

Medication Abortion in the First Trimester



SUPREME COURT OVERTURNS ROE V. WADE

LIVE

Ruling Ends Constitutional Right to Abortion

- The decision, eliminating the right to abortion after almost 50 years, will lead to all but total bans on the procedure in about half of the states.
- It will also be one of the chief legacies of former President Trump: All three of his appointees were in the majority ruling. Here's the latest.

This is what the justices in the majority opinion said about Roe during their



"We therefore hold that the Constitution does not confer a right to abortion. Roe and Casey must be overruled, and the authority to regulate abortion must be returned to the people and their elected representatives."

— Dobbs v. Jackson Women's Health Organization

Read the majority decision overruling Roe, written by Justice Samuel Alito.

The New York Times is tracking the status of abortion laws in each state.

Take our quiz to test your knowledge!



Impact

Approach

Medications

Follow-up

Side Effects

Cases

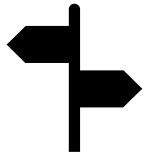
Stepwise Approach to Medication Abortion



Confirm pregnancy and gestational age

Considered up to 77 d (11 wk) of gestation

- First day of LMP
- $\pm$ Ultrasound
 - Unknown LMP (or during hormonal contraception)
 - Risk for ectopic pregnancy



Choose a medication regimen

- Mifepristone + misoprostol (95-98% effective)
- Misoprostol alone (76-88% effective)



Assess for medical contraindications

- Bleeding disorders, anticoagulation
- Ectopic pregnancy
- IUD
- Hgb <9.5 g/dL (relative)

Medication Specific

- Mifepristone: steroids, adrenal failure
- Misoprostol: porphyria



Obtain consent

- Benefits, risks, alternatives
- Provider agreement
- Medication information handout
- After-care instructions

Medications for Abortion

Impact

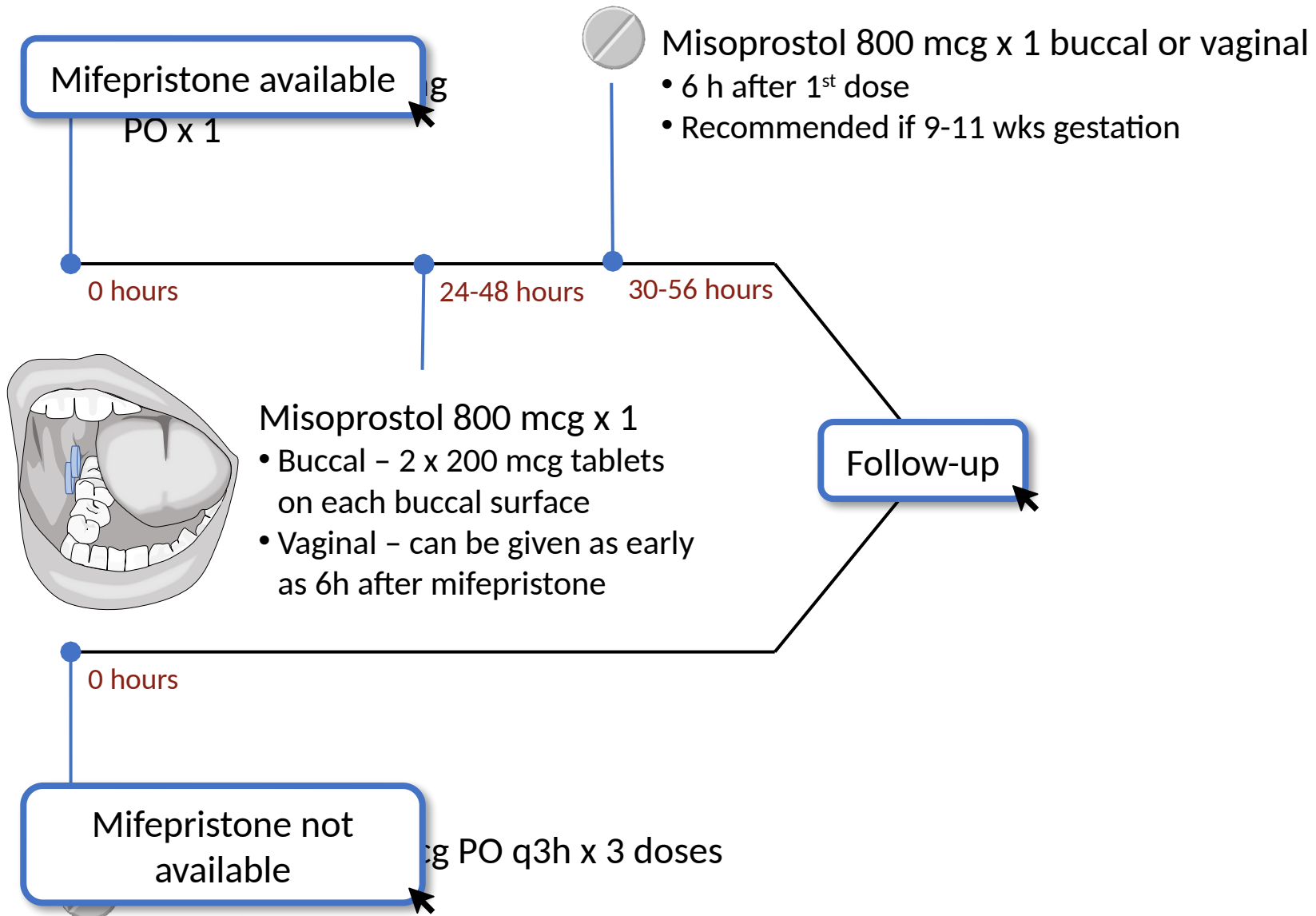
Approach

Medications

Follow-up

Side Effects

Cases



Abortion Follow-up

Impact

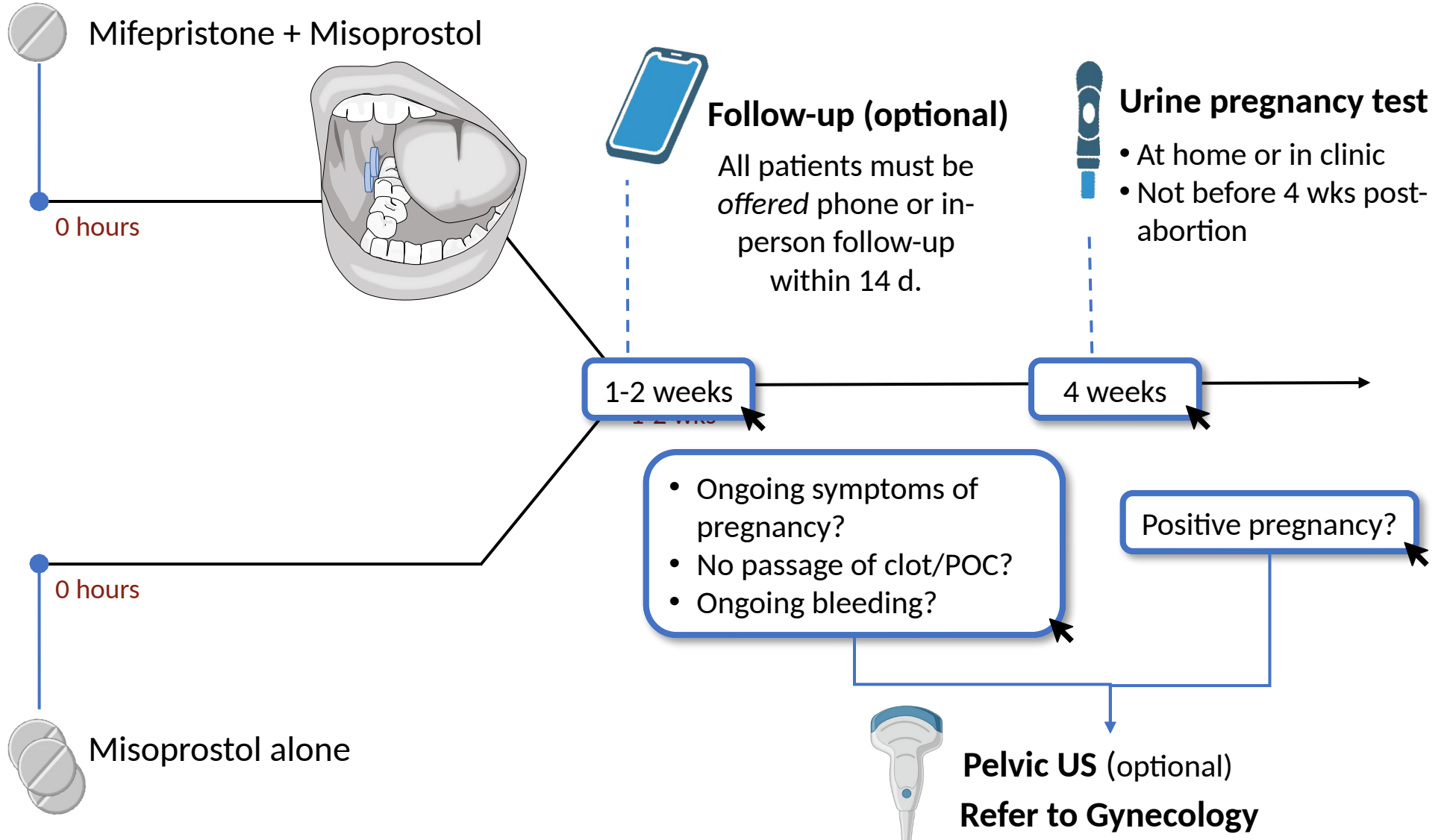
Approach

Medications

Follow-up

Side Effects

Cases



Impact

Approach

Medications

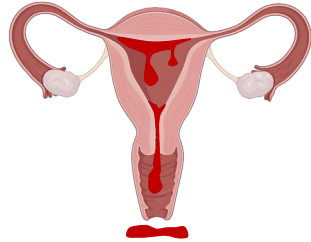
Follow-up

Side Effects

Cases

Side Effects and Complications

Expected Side Effects



Cramping/
bleeding



Fevers/ malaise

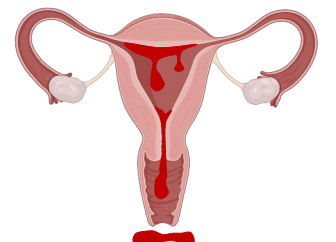


GI symptoms

Complications



Incomplete/
failed abortion



Prolonged/heavy
bleeding



Intrauterine
infection

- F
 - S
 - H
 - V
 - R
- abortion

f...

Impact

Approach

Medications

Follow-up

Side Effects

Cases

1

2

Case 1a

A 30-year-old transgender man (they/them) with a uterus presents for a missed period. Their LMP was 6 weeks ago. They are not on hormonal contraception and are worried about pregnancy.

What is your next step in evaluation?

Confirm pregnancy with **Urine hcg**

...Urine pregnancy test is positive.

What is the gestational age?

EGA is **6 weeks**.

How do you counsel on patient's choice regarding their pregnancy?

Review options including carrying pregnancy, adoption and terminating pregnancy. Provide additional resources.

What resources or restrictions are in effect in your state?

Take a moment to familiarize yourself with waiting periods and restrictions from the Guttmacher Institute.

Case 1b

They decide they would like to proceed with medication abortion. They have no medical problems and are not on any medications.

Do they have any contraindications to medical abortion?

No. Contraindications include:

- Any medical abortion: ectopic pregnancy or IUD in place
- Mifepristone: corticosteroids or known adrenal failure
- Misoprostol: porphyria

What history might lead you to suspect ectopic pregnancy?

- **High risk:** prior ectopic pregnancy, tubal surgery or pathology, IUD use, and use of in-vitro fertilization for the current pregnancy

Case 1c

You prescribe mifepristone + misoprostol x 1 dose.

How do you counsel them on side effects?

- Severe cramping and bleeding that is heavier than menses, typically occurs 2.5-4 hours after misoprostol dose and lasts 1-24 hours though rarely 24h
- Nausea, vomiting, diarrhea
- Fever for <24h, hot flushes, chills.

...They call back with heavy bleeding for the past 5 hours, soaking through 4 pads an hour for the past 2 hours. They deny light-headedness or dyspnea.

What is your next step in management?

This is heavier than expected bleeding (>2 pads/1h x 2 h). They should be evaluated in the ED. This is an extremely rare complication in <1%.



Impact

Approach

Medications

Follow-up

Side Effects

Cases

1

2

Case 2a

A 21-year-old woman with history of SLE on prednisone 5 mg daily seeks contraception counseling. She is interested in having children in the distant future but wonders how her condition will affect pregnancy.

According to the CDC, what medical conditions are associated with increased risk for adverse health events as a result of pregnancy?

- SLE
- Insulin-dependent diabetes with complications
- Uncontrolled HTN
- Heart disease (valvular, ischemic, peripartum)
- Bariatric surgery within the past 2 years
- Solid organ transplant within the past 2 years (see [CDC website](#) for full list)

...She is interested in the copper IUD. Her LMP was 9-11 weeks ago but she often has irregular periods.

What is the next step in evaluation?

Urine pregnancy test

Case 2b

Urine HCG is positive. She wishes to have a medication abortion.

What is the next step in evaluation?

Because she is unclear on her LMP, **ultrasound** is needed to confirm gestational age <11 weeks

...Transabdominal ultrasound demonstrates EGA of 9 weeks.

What medication(s) do you prescribe?

Misoprostol 800 mcg x 3 due to EGA > 9 weeks. Mifepristone is contraindicated due to her corticosteroid use.

...The patient anticipates undergoing future pregnancy.

Would you recommend Rh testing prior to abortion?

No. Rh testing is no longer recommended for abortion under 12 weeks, even for those who are known to be Rh neg as there is minimal risk of enough maternal-fetal blood mixing to induce alloimmunization.

Case 2c

How will you counsel her on the risk of incomplete abortion?

She has higher risk of incomplete abortion.

- Misoprostol alone is associated with higher risk of incomplete abortion (78-93% success with misoprostol-only abortions compared with 93-95% with the combination regimen) [2019 meta-analysis](#).
- Risk of incomplete abortion also increases as the EGA approaches 10 weeks.

...During a telephone visit 1 week later, she continues to have morning sickness and breast tenderness.

What are the next steps in management?

If <11 weeks EGA, patients may take an extra dose of misoprostol 1 week after the first for ongoing symptoms of pregnancy. Alternatively, she may be referred now for surgical abortion.

Access & Regulatory Update (2023–2026) — clinical regimen unchanged

- Jan 2023 — FDA modified the mifepristone REMS: in-person dispensing requirement removed; certified retail and mail-order pharmacies may dispense with a prescription from a certified prescriber (CVS/Walgreens certified, Mar 2024).
- Jun 13, 2024 — FDA v. Alliance for Hippocratic Medicine: U.S. Supreme Court (9–0) dismissed the challenge for lack of standing; FDA approval and the 2016/2021–23 REMS changes stand.
- May 2026 — Louisiana v. FDA: Fifth Circuit (May 1, 2026) ordered a return to in-person-only dispensing; the Supreme Court stayed that order May 14, 2026 (Danco Labs v. Louisiana), preserving telehealth prescribing + mail/pharmacy dispensing while litigation continues.
- **What this means for this talk: the clinical teaching (regimens, contraindications, follow-up) is unchanged. The ACCESS landscape remains in flux — verify current federal status and your state law (Guttmacher Institute) before prescribing.**
 - Sources: FDA REMS (Jan 2023); SCOTUS 23-235 (Jun 2024); SCOTUS 25A1207 (May 2026); Guttmacher Institute state-policy tracker.

Take-Home Points

- Before prescribing: confirm pregnancy (urine hCG), estimate gestational age from LMP ($\pm$ ultrasound if LMP unknown, hormonal contraception, or ectopic risk), screen contraindications, and check your state law / mandatory waiting periods.
- Medication abortion is used up to 11 wk (77 d); beyond 10 wk is off-label. Mifepristone 200 mg PO, then misoprostol 800 mcg buccal/vaginal 24–48 h later (95–98% effective); add a 2nd misoprostol dose if 9–11 wk.
- If mifepristone is unavailable or contraindicated: misoprostol 800 mcg q3h $\times$ 3 doses (less effective, ~76–93%).
- Contraindications: ectopic pregnancy, IUD in place, coagulopathy/anticoagulation (any regimen); chronic corticosteroids/adrenal failure (mifepristone); porphyria (misoprostol).
- Expected: cramping + bleeding heavier than menses (~2.5–4 h after misoprostol), GI symptoms, fever/chills. Seek care: >2 pads/h $\times$ 2 h, prolonged pelvic pain, abnormal discharge, fever >24 h.
- Offer follow-up within 14 d; urine pregnancy test no earlier than 4 wk. Rh testing NOT recommended <12 wk. No effect on future fertility.

Attribution & Changelog

- Original: "Medical Abortion in the First Trimester," TeachIM.org, June 2023 — deQuillfeldt, Asare, Cleeves, Soiseth, Ramaswami, Kendall, Lazorwitz, Belmonte; Section Ed. McClintock; Exec. Ed. Zhang.
 - https://teachim.org/teaching_material/medical-abortion-in-the-first-trimester/ — CC BY-NC 4.0 (attribution required, non-commercial). Optimized June 2026.
- **Corrections made to this copy (animations and original slides preserved):**
 - Case 2b: "Misoprostol 800 mg" → "800 mcg" (dose unit; matches Medications slide) and "EGD" → "EGA".
 - Case 1c: "heaving bleeding" → "heavy bleeding"; Case 1a: side-effect onset "2.4-4" → "2.5-4 hours" (matches teaching script).
 - Approach slide: relative-contraindication anemia threshold "Hgb <9 g/dL" → "<9.5 g/dL" (matches the talk's own script/notes — flagged for author confirmation).
- Appended slides: Access & Regulatory Update (2023–2026), Take-Home Points, this attribution slide. No original content removed.
- Companion files: Pre-and-Post-Test-Medical-Abortion.docx (pre/post quiz). Package: Medical_Abortion_Lesson_Package.html, Medical_Abortion_OnePager.pdf, Medical_Abortion_Anki_Cards.txt.
- Educational use only — not medical advice. Verify current law and institutional policy before prescribing.